

MEDCORE HOSPITALS & HEALTHCARE SERVICES PVT. LTD.

LEGAL & COMPLIANCE DEPARTMENT

PATIENT CONSENT POLICY

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Document Owner	Head – Legal & Compliance	Policy Sponsor	Chief Operating Officer
Applies To	All MedCore hospitals, clinics, employees, consultants and authorized service partners	Classification	Internal – Controlled Document

1. Purpose and Policy Statement

This policy establishes the minimum requirements for obtaining, recording, verifying, maintaining and managing patient consent across MedCore Hospitals & Healthcare Services Pvt. Ltd.

Consent is a documented process, not merely a signed form. Staff must provide information appropriate to the activity, allow reasonable opportunity for questions, confirm understanding of material information, and record the outcome in the approved system or form.

Where a specific procedure, service line or local operating procedure imposes a more detailed requirement, that procedure must be followed in addition to this policy. Questions about interpretation should be escalated to the relevant clinical lead and, where appropriate, Legal & Compliance.

1.1 Scope

The policy covers clinical procedures, diagnostic activities, treatment-related decisions, release or sharing of patient information, photography and recording, approved programs, and other activities for which MedCore requires documented authorization.

1.2 Core principles

Informed: material information is communicated in a reasonably understandable manner. Voluntary: consent is not obtained through coercion or inappropriate pressure. Specific: authorization relates to the identified activity or purpose. Documented: the decision is captured in the approved record. Reviewable: consent status is revisited when circumstances change or consent is withdrawn.

2. Definitions and Roles

Consent means a voluntary authorization provided after relevant information has been communicated and an opportunity for questions has been provided. A consent record is the approved electronic or paper record containing the decision and supporting details.

2.1 Key definitions

Term	Definition
Authorized Decision-Maker	A person permitted under the applicable MedCore workflow to make or communicate a decision on behalf of a patient.
Material Information	Information that could reasonably affect the decision, including relevant purpose, risks, alternatives and operational implications.
Withdrawal	A communicated decision to stop or limit a previously authorized activity, subject to applicable operational procedures.
Witness	An authorized staff member who confirms that the consent interaction or signature occurred as recorded.

2.2 Responsibility matrix

Role	Responsibility
Treating / Responsible Clinician	Explain relevant procedure-specific information, answer questions within scope, assess the consent process and document the decision.
Nursing / Clinical Staff	Verify consent status before applicable activities, identify gaps and escalate concerns.
Patient Services	Capture assigned administrative consent, verify identity and route exceptions.
Medical Records	Maintain record integrity, retrieval, retention workflows and approved corrections.
Legal & Compliance	Interpret policy requirements, manage escalations and conduct compliance reviews.

3. Consent Categories

Different activities require different consent routes. Staff must not assume that one general consent record authorizes every subsequent activity.

Category	Examples	Minimum control
Routine Service Consent	Routine diagnostic or service activity where a specific consent step is prescribed.	Approved workflow; identity verification; status recorded before activity.
Procedure-Specific Consent	Procedures or interventions requiring detailed explanation and authorization.	Procedure-specific form or electronic workflow and responsible clinician identification.
Information Disclosure Consent	Release of records or specified information to an external recipient.	Recipient, purpose, scope and authorization status documented before release.
Photography / Recording	Clinical images, recordings or approved educational/operational capture.	Purpose and permitted use documented through the relevant process.
Program / Research-Related	Participation in approved programs requiring a dedicated consent pathway.	Use the approved program-specific process.

3.1 Consent validity checks

Check	Required control
Identity	Confirm the correct patient using approved identifiers.
Authority	Confirm the person providing consent is authorized for the decision.
Information	Confirm required information was communicated.
Understanding	Allow reasonable opportunity for questions and clarification.
Documentation	Ensure the record is complete and attributable.

3.2 General consent and secondary use

General service consent does not automatically authorize unrelated disclosure, publication, recording, promotional use or other secondary activity unless the relevant approved process explicitly provides that authorization.

4. Standard Consent Workflow

The default workflow is designed to make the consent decision traceable from the initial request through final record storage.

Step	Action	Control point
1	Identify the activity requiring consent.	Confirm the correct form or electronic workflow.
2	Verify patient identity and decision-maker authority where relevant.	Use approved identifiers and authority checks.
3	Provide material information in an understandable manner.	Allow questions and clarification.
4	Record the patient's decision.	Use the approved electronic or paper record.
5	Obtain required witness, clinician or electronic authentication.	Ensure the entry is attributable.
6	Verify consent status before the activity begins.	If unresolved, pause and escalate where appropriate.
7	Store and protect the record.	Apply role-based access and record-management controls.

4.1 Minimum consent record

The record should contain the patient identifier, activity or purpose, decision status, date and time, consent provider, responsible staff member where required, relevant notes and authentication.

4.2 Pre-activity verification

Before an applicable activity begins, the responsible team should confirm that the correct consent is active, corresponds to the planned activity and has not been withdrawn or superseded.

5. Special Circumstances

5.1 Authorized decision-makers

Where a patient cannot independently provide the required authorization, staff must verify the applicable authorized decision-maker through the approved MedCore workflow. The record should identify the relationship or authority basis where required.

5.2 Language and communication support

Where a patient cannot reasonably understand the information provided, staff should use an approved language or communication-support route. Informal interpretation should not replace an approved process where one is required.

5.3 Urgent or emergency circumstances

If an urgent circumstance prevents completion of the standard process, the responsible clinical team must follow the applicable emergency procedure and document the circumstances, actions taken and subsequent communication or documentation.

5.4 Refusal and withdrawal

A patient may decline an activity. Staff must record the refusal using the approved status and communicate relevant operational consequences. Where withdrawal is applicable, it must be recorded promptly and communicated to the team responsible for the affected activity.

5.5 Digital and remote consent

Digital consent may be used only through approved systems. Electronic records should preserve attributable user identity, date/time and the consent content or version presented where the system supports such controls.

6. Consent Documentation Standards

Consent records are part of the controlled patient record. Documentation must be factual, concise and contemporaneous.

Do	Do not
Record the actual consent status.	Do not assume consent from silence where explicit authorization is required.
Use the current approved form or workflow.	Do not use an obsolete local template when the controlled version is available.
Document material clarification where required.	Do not enter speculation or personal judgments.
Use the approved correction process.	Do not overwrite, backdate or delete original entries outside the approved process.
Record withdrawal or refusal promptly.	Do not mischaracterize the patient's decision.

6.1 Missing, inconsistent or late records

If consent documentation is missing, unclear or inconsistent with the planned activity, staff must check approved record sources and seek clarification from the responsible team before proceeding where operationally appropriate.

6.2 Amendments and retention

Corrections must preserve the integrity of the original record. Electronic audit trails or approved correction procedures must be used. Consent records must be retained according to the applicable MedCore medical-record retention schedule.

7. Access, Confidentiality and Information Sharing

Consent information is accessible only to personnel with a legitimate operational need and appropriate system permissions. Access should be limited to the information necessary for assigned responsibilities.

7.1 Role-based access

Role group	Typical access	Restriction
Clinical Team	Relevant consent status and procedure documentation.	Assigned patients and legitimate care responsibilities.
Patient Services	Administrative consent status for assigned workflows.	No unrestricted access to unrelated clinical details.
Medical Records	Record maintenance and authorized retrieval.	Access governed by role.
Legal & Compliance	Records required for audit, investigation or dispute.	Authorized review purposes only.
IT Support	Technical metadata required for system support.	Content access only when specifically authorized and necessary.

7.2 External disclosure

Before releasing consent-related information externally, staff must confirm the applicable authorization, recipient, scope and approved disclosure route. Requests outside routine workflows must be escalated to Legal & Compliance or the designated records authority.

7.3 Auditability

Systems supporting consent records should maintain appropriate access and activity logs. Suspected inappropriate access, alteration or disclosure must be reported through the applicable incident-management process.

8. Exceptions, Escalation and Compliance Monitoring

Exceptions should be managed through documented escalation rather than informal approval. Unusual situations must remain visible, attributable and reviewable.

8.1 Escalation matrix

Scenario	Immediate action	Escalate to
Consent record unavailable	Verify approved sources and pause where operationally appropriate.	Responsible Clinician / Medical Records
Authority unclear	Do not rely on assumptions; verify authority.	Department Head / Legal & Compliance
Patient disputes consent	Preserve the record and document the concern.	Clinical / Patient Services / Legal
Suspected unauthorized access	Preserve evidence and limit further access.	Information Security / Legal

8.2 Compliance monitoring

Legal & Compliance may periodically sample consent records for completeness, timeliness, attribution, exception handling, withdrawal/refusal documentation and access activity.

8.3 Corrective action

Recurring gaps may result in staff retraining, workflow modification, form revision, system configuration changes or targeted record review. Department Heads are responsible for tracking assigned actions.

9. Training, Review and Document Governance

Personnel whose duties involve consent must understand the relevant workflow and escalation route. New staff should receive applicable training before independently performing consent-related responsibilities.

This policy is reviewed at least annually and earlier after material operational, system or requirements changes, significant incidents or audit findings.

9.1 Version history

Version	Effective date	Change summary
1.0	01 Apr 2024	Initial controlled release.
1.1	15 Nov 2024	Clarified documentation and escalation requirements.
1.2	10 Aug 2025	Updated digital consent and access-control provisions.
2.0	01 Apr 2026	Revised workflow, roles, exceptions and monitoring framework.

10. Operational Decision Guide and FAQs

Before an applicable activity, confirm the correct consent route, patient or decision-maker identity, required information, clear documentation, and any refusal or withdrawal status. If a required control is unresolved, use the applicable escalation route.

Frequently Asked Questions

Q1. Can a general form be used for every procedure? No. Use the procedure-specific workflow when required.

Q2. What if the patient says they did not understand? Pause where appropriate, provide clarification and document the interaction.

Q3. Can consent be withdrawn? Where applicable, record and communicate the withdrawal through the approved workflow.

Q4. What if the consent record is missing? Do not assume consent exists; check approved sources and escalate where appropriate.

Q5. Who handles unusual disputes? Begin with the responsible clinical/service team and escalate to Department leadership and Legal & Compliance when outside routine procedure.

Related controlled documents

Data Privacy & Confidentiality Policy; Medical Record Access Policy; Patient Complaint & Grievance Policy; Information Security Policy.